

*Carefully read the instructions before use. If necessary,
ask your physician for further information.
This drug should be used under prescription only.*

Product Name

Valthotan F.C. Tablet 80mg
Valthotan F.C. Tablet 160mg

Name and Strength of Active Substance(s)

Valsartan 80mg
Valsartan 160mg

Product Description

For Valthotan F.C. Tablet 80mg, it occurs as a pale red film-coated, round biconvex tablet; engraved with “1007” on one surface and “S || D” script and score on the other surface.
For Valthotan F.C. Tablet 160mg, it occurs as a dark yellow film-coated tablet, are oval-shaped with beveled edges, and engraved with “S || D” script and score on one surface and engraved with “736” script on the other surface.

Pharmacodynamics

Angiotensin II antagonist.

The active hormone of the renin-angiotensin-aldosterone system (RAAS) is angiotensin II, which is formed from angiotensin I through ACE. Angiotensin II binds to specific receptors located in the cell membranes of various tissues. It has a wide variety of physiological effects, including in particular both direct and indirect involvement in the regulation of blood pressure. As a potent vasoconstrictor, angiotensin II exerts a direct pressor response. In addition, it promotes sodium retention and stimulation of aldosterone secretion.

Valsartan is an orally active, potent and specific angiotensin II (Ang II) receptor antagonist. It acts selectively on the AT₁ receptor subtype, which is responsible for the known actions of angiotensin II. The increased plasma levels of Ang II following AT₁ receptor blockade with valsartan may stimulate the unblocked AT₂ receptor, which appears to counterbalance the effect of the AT₁ receptor. Valsartan does not exhibit any partial agonist activity at the AT₁ receptor and has much (about 20,000 fold) greater affinity for the AT₁ receptor than for the AT₂ receptor.

Valsartan does not inhibit ACE, also known as kininase II, which converts Ang I to Ang II and degrades bradykinin. Since there is no effect on ACE and no potentiation of bradykinin or substance P, angiotensin II antagonists are unlikely to be associated with cough. Valsartan does not bind to or block other hormone receptors or ion channels known to be important in cardiovascular regulation.

Administration of valsartan to patients with hypertension results in reduction of blood pressure without affecting pulse rate.

In most patients, after administration of a single oral dose, onset of antihypertensive activity occurs within 2 hours, and the peak reduction of blood pressure is achieved within 4-6 hours. The antihypertensive effect persists over 24 hours after dosing. During repeated dosing, the maximum reduction in blood pressure with any dose is generally attained within 2-4 weeks and is sustained during long-term therapy. Combined with hydrochlorothiazide, a significant additional reduction in blood pressure is achieved.

Abrupt withdrawal of valsartan has not been associated with rebound hypertension or other adverse clinical events.

Pharmacokinetics

Absorption of valsartan after oral administration is rapid, although the amount absorbed varies widely. Mean absolute bioavailability for valsartan is 23%. Valsartan shows multiexponential decay kinetics (t_{1/2} alpha <1 hour and t_{1/2} beta about 9 hours).

The pharmacokinetics of valsartan are linear in the dose range tested. There is no change in the kinetics of valsartan on repeated administration, and little accumulation when dosed once daily. Plasma concentrations were observed to be similar in males and females.

Valsartan is highly bound to serum protein (94-97%), mainly serum albumin. Steady-state volume of distribution is low (about 17 L). Plasma clearance is relatively slow (about 2 L/h) when compared with hepatic blood flow (about 30 L/h). Of the absorbed dose of valsartan, 70% is excreted in the faeces and 30% in the urine, mainly as unchanged compound.

When valsartan is given with food, the area under the plasma concentration curve (AUC) of valsartan is reduced by 48%, although from about 8 hours post dosing, plasma valsartan concentrations are similar for the fed and fasted group. However, this reduction in AUC is not accompanied by a clinically significant reduction in the therapeutic effect, and valsartan can therefore be given either with or without food.

The average time to peak concentration and elimination half-life of valsartan in heart failure patients are similar to that observed in healthy volunteers. AUC and C_{max} values of valsartan increase linearly and are almost proportional with increasing dose over the clinical dosing range (40 to 160 mg twice a day). The average accumulation factor is about 1.7. The apparent clearance of valsartan following oral administration is approximately 4.5 L/h. Age does not affect the apparent clearance in heart failure patients.

Elderly

A somewhat higher systemic exposure to valsartan was observed in some elderly subjects compared to young subjects; however, this has not been shown to have any clinical significance.

Impaired renal function

As expected for a compound where renal clearance accounts for only 30% of total plasma clearance, no correlation was seen between renal function and systemic exposure to valsartan. Dose adjustment is therefore not required in patients with renal impairment. No studies have been performed in patients undergoing dialysis. However, valsartan is highly bound to plasma protein and is unlikely to be removed by dialysis.

Hepatic impairment

About 70% of the absorbed dose is excreted in the bile mainly as unchanged compound. Valsartan does not undergo extensive biotransformation, and, as expected, systemic exposure to valsartan is not correlated with the degree of liver dysfunction. No dose adjustment for valsartan is therefore necessary in patients with hepatic insufficiency of non-biliary origin and without cholestasis. The AUC with valsartan has been observed to approximately double in patients with biliary cirrhosis or biliary obstruction.

Indication

Hypertension

Treatment of hypertension.

Heart failure

Treatment of heart failure (NYHA class II-IV) in patients receiving usual therapy (such as diuretics, digitalis) who are intolerant to ACE inhibitors. Valsartan improves morbidity in these patients, primarily via reduction in hospitalisation for heart failure. Valsartan also slows the progression of heart failure, improves NYHA functional class, ejection fraction and signs and symptoms of heart failure and improves quality of life versus placebo.

Post-myocardial infarction

Valsartan is indicated to improve survival following myocardial infarction in clinically stable

patients with signs, symptoms or radiological evidence of left ventricular failure and/or with left ventricular systolic dysfunction.

Recommended Dosage

Hypertension

The recommended dose of valsartan is 80 mg or 160 mg film-coated tablet once daily, irrespective of race, age, or gender. The antihypertensive effect is substantially present within 2 weeks and maximal effects are seen after 4 weeks. In patients whose blood pressure is not adequately controlled, the daily dose may be increased to 320 mg film-coated tablet, or a diuretic may be added. Valsartan may also be administered with other antihypertensive agents.

Heart failure

The recommended starting dose of valsartan is 40 mg film-coated tablet twice daily. Up titration to 80 mg and 160 mg twice daily should be done to the highest dose tolerated by the patient. Consideration should be given to reducing the dose of concomitant diuretics. The maximum daily dose administered is 320 mg in divided doses. Evaluation of patients with heart failure should always include assessment of renal function.

Post-myocardial infarction

Therapy may be initiated as early as 12 hours after a myocardial infarction. After an initial dose of 20 mg twice daily, valsartan therapy should be titrated to 40 mg, 80 mg, and 160 mg film-coated tablet twice daily over the next few weeks. The starting dose is provided by the 40 mg divisible tablet.

The target maximum dose is 160 mg twice daily. In general, it is recommended that patients achieve a dose level of 80 mg twice daily by two weeks after treatment initiation and that the target maximum dose be achieved by three months, based on the patient's tolerability to valsartan during titration. If symptomatic hypotension or renal dysfunction occur, consideration should be given to a dosage reduction.

Valsartan may be used in patients treated with other post-myocardial infarction therapies, e.g. thrombolytics, acetylsalicylic acid, beta blockers or statins.

Evaluation of post-myocardial infarction patients should always include assessment of renal function.

For all indications, no dosage adjustment is required for patients with renal impairment or for patients with hepatic insufficiency of non-biliary origin and without cholestasis.

Use in children and adolescents

The safety and efficacy of Valsartan have not been established in children and adolescents (below the age of 18 years).

Method of administration

Valsartan tablets may be taken independently of a meal and should be administered with water.

Route of Administration

Oral

Contraindications

Known hypersensitivity to valsartan or to any of the excipients of valsartan.

Pregnancy

Concomitant use of angiotensin receptor antagonists (ARBs) - including Valsartan - or of angiotension-converting enzyme inhibitors (ACEIs) with aliskiren in patients with Type 2 diabetes.

Warnings and Precautions

Sodium- and/or volume-depleted patients

In severely sodium-depleted and/or volume-depleted patients, such as those receiving high doses of diuretics, symptomatic hypotension may occur in rare cases after initiation of therapy with valsartan. Sodium and/or volume depletion should be corrected before starting treatment with valsartan, for example by reducing the diuretic dose. If hypotension occurs, the patient should be placed in the supine position and, if necessary, given an i.v. infusion of normal saline. Treatment can be continued once blood pressure has been stabilised.

Renal artery stenosis

Short-term administration of valsartan to twelve patients with renovascular hypertension secondary to unilateral renal artery stenosis did not induce any significant changes in renal haemodynamics, serum creatinine, or blood urea nitrogen (BUN). However, since other drugs that affect the renin-angiotensin-aldosterone system (RAAS) may increase blood urea and serum creatinine in patients with bilateral or unilateral renal artery stenosis, monitoring of both parameters is recommended as a safety measure.

Impaired renal function

No dosage adjustment is required for patients with renal impairment. However, no data is available for severe cases (creatinine clearance <10 mL/min.), and caution is therefore advised.

The use of ARBs - including Valsartan - or of ACEIs with aliskiren should be avoided in patients with severe renal impairment (GFR<30ml/min).

Hepatic impairment

No dosage adjustment is required for patients with hepatic insufficiency. Valsartan is mostly eliminated unchanged in the bile, and patients with biliary obstructive disorders showed lower valsartan clearance. Particular caution should be exercised when administering valsartan to patients with biliary obstructive disorders.

Heart failure / Post-myocardial infarction

Use of valsartan in patients with heart failure or post-myocardial infarction commonly results in some reduction in blood pressure, but discontinuation of valsartan therapy because of continuing symptomatic hypotension is not usually necessary provided dosing instructions are followed.

Caution should be observed when initiating therapy in patients with heart failure or post-myocardial infarction.

As a consequence of the inhibition of the RAAS, changes in renal function may be anticipated in susceptible individuals. In patients with severe heart failure whose renal function may depend on the activity of the RAAS, treatment with ACE inhibitors or angiotensin receptor antagonists has been associated with oliguria and/or progressive azotaemia and (rarely) with acute renal failure and/or death. Evaluation of patients with heart failure or post-myocardial infarction should always include assessment of renal function.

In patients with heart failure, caution should be observed with the triple combination of an ACE inhibitor, a beta blocker and valsartan.

Angioedema

Angioedema, including swelling of the larynx and glottis, causing airway obstruction and/or swelling of the face, lips, pharynx, and/or tongue has been reported in patients treated with valsartan; some of these patients previously experienced angioedema with other drugs including ACE inhibitors. Valsartan should be immediately discontinued in patients who develop angioedema, and Valsartan should not be re-administered.

Dual Blockade of the Renin-Angiotensin System (RAS)

Caution is required while co-administering ARBs, including Valsartan, with other agents

